

Patient Name: AMRUTHA C	Age/Gender: 23Y/F	Patient ID: 2825718
Study Date: N/A	Ref. Doctor: john	Accession No: 65845
Reported Date: 16/2/2026, 12:17:56 pm	Modality: CR	Body Part: CHEST

CR CHEST REPORT

History:

The trachea is central in position. The cardiac silhouette is within normal limits. Both lung fields are well expanded and appear clear. No focal consolidation, cavitary lesion, or nodular opacity is identified. There is no evidence of pleural effusion or pneumothorax. The costophrenic angles are clear bilaterally. The mediastinum does not appear widened. Bilateral hilar regions are unremarkable. Bronchovascular markings are within normal limits. The diaphragmatic domes are well defined. The visualized bony thorax including ribs, clavicles, and scapulae appears intact without any obvious abnormality. No soft tissue abnormality is seen. Overall radiographic appearance is within normal limits. The patient presents with complaints of cough for the past five days, associated with mild fever and throat irritation. There is occasional shortness of breath but no history of chest pain or hemoptysis. The patient denies any significant weight loss, night sweats, or recent travel history. There is no known contact with tuberculosis patients and no prior history of pulmonary tuberculosis. No history of asthma, chronic lung disease, or previous chest surgery is reported. The patient is a non-smoker / occasional smoker with no known occupational exposure to dust or chemicals. There is no significant past medical or cardiac history. The examination has been advised for clinical evaluation and correlation.

Findings:

The trachea is central in position. The cardiac silhouette is within normal limits. Both lung fields are well expanded and appear clear. No focal consolidation, cavitary lesion, or nodular opacity is identified. There is no evidence of pleural effusion or pneumothorax. The costophrenic angles are clear bilaterally. The mediastinum does not appear widened. Bilateral hilar regions are unremarkable. Bronchovascular markings are within normal limits. The diaphragmatic domes are well defined. The visualized bony thorax including ribs, clavicles, and scapulae appears intact without any obvious abnormality. No soft tissue abnormality is seen. Overall radiographic appearance is within normal limits. No radiographic evidence of active cardiopulmonary pathology is identified. There is no focal consolidation, pleural effusion, pneumothorax, or mass lesion. Cardiac size appears normal. The mediastinum and hilar structures are unremarkable. The lung fields are clear bilaterally. No radiological signs of active pulmonary tuberculosis are noted. The visualized bony thorax appears normal. Impression: Normal Chest Radiograph. Clinical correlation is advised. Follow-up may be considered if symptoms persist or worsen. There is no evidence of pleural effusion or pneumothorax. The costophrenic angles are clear bilaterally. The mediastinum does not appear widened. Bilateral hilar regions are unremarkable. Bronchovascular markings are within normal limits.

Conclusion:

No radiographic evidence of active cardiopulmonary pathology is identified. There is no focal consolidation, pleural effusion, pneumothorax, or mass lesion. Cardiac size appears normal. The mediastinum and hilar structures are unremarkable. The lung fields are clear bilaterally. No radiological signs of active pulmonary tuberculosis are noted. The visualized bony thorax appears normal. Impression: Normal Chest Radiograph. Clinical correlation is advised. Follow-up may be considered if symptoms persist or worsen. The trachea is central in position. The cardiac silhouette is within normal limits. Both lung fields are well expanded and appear clear. No focal consolidation, cavitary lesion, or nodular opacity is identified. There is no evidence of pleural effusion or pneumothorax. The costophrenic angles are clear bilaterally. The mediastinum does not appear widened. Bilateral hilar regions are unremarkable. Bronchovascular markings are within normal limits. The diaphragmatic domes are well defined. The visualized bony thorax including ribs, clavicles, and scapulae appears intact without any obvious abnormality. No soft tissue abnormality is seen. Overall radiographic appearance is within normal limits.

Key Images:



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